

Premier — Semaglutide with Glycine

Weight Loss & Metabolic Health · Provider Reference

KORB Health Medical Texas PA · korb-glpi-data.js v2.17 · built 2026-09-15

Everything needed to prescribe Premier Semaglutide with Glycine, complete on its own. Values are copied literally into Tebra — do not paraphrase, and do not adjust quantity, refill or days supply.

Premier Pharmacy at a glance

Status	Active
Preferred states	TX, NV, AZ, FL, IL, MD, MO, NJ, NY, OH
Ships to	AZ, CO, CT, DC, DE, FL, GA, IL, KS, KY, LA, MD, ME, MI, MO, MS, MT, NC, ND, NE, NJ, NM, NV, NY, OH, OK, OR, PA, RI, SD, TN, TX, UT, VA, VT, WI, WV, WY
CANNOT ship to	AK, AL, AR, CA, HI, IA, ID, IN, MA, MN, NH, SC, WA
Order via	Tebra Compound
Billing	Bill to KORB Health Group, ship to patient
Dispensing address	Premier Pharmacy, 2425 Babcock Rd, Ste 108A, San Antonio, TX 78229

Compounded preparation

KORB dispenses a COMPOUNDED preparation, not the FDA-approved branded product. The molecule is approved; this specific formulation is not, and compounded products are not reviewed by the FDA for safety, efficacy or quality. Counsel accordingly and document that the distinction was explained.

Before you prescribe

- All Premier programs offer a 4-week and an 8-week option — semaglutide, semaglutide with glycine, and tirzepatide alike.
- All Premier compounds carry a 90-day BUD as of 2026-08. There is no longer a difference between the glycine and non-glycine products on this.
- 8-week programs ship the full eight weeks of medication and supplies in a single initial shipment. No refill and no second shipment.

- Vials remain 28 days from first use regardless of BUD. On lower doses this produces overage. Counsel the patient to discard at 28 days.

Vials dispensed

Dose	Units	Concentration	4-week	8-week
0.3 mg	10 units	Semaglutide 3 mg / B-12 0.5 mg / Glycine 5 mg per mL	1 ml vial	1 ml x 2 vials
0.6 mg	20 units	Semaglutide 3 mg / B-12 0.5 mg / Glycine 5 mg per mL	1 ml vial	1 ml x 2 vials
1.2 mg	40 units	Semaglutide 3 mg / B-12 0.5 mg / Glycine 5 mg per mL	1 ml x 2 vials	1 ml x 4 vials
1.8 mg	60 units	Semaglutide 3 mg / B-12 0.5 mg / Glycine 5 mg per mL	1 ml x 3 vials	1 ml x 5 vials
2.7 mg	90 units	Semaglutide 3 mg / B-12 0.5 mg / Glycine 5 mg per mL	1 ml x 4 vials	1 ml x 8 vials

Tebra — copy these values literally

Do not paraphrase, and do not adjust quantity, refill or days supply.

Premier — Semaglutide / B-12 / Glycine

Tebra Compound

Only from Premier Pharmacy — this product

The glycine formulation is only available from Premier. A patient moving to a state Premier cannot ship to must move to another pharmacy and switch to semaglutide WITHOUT glycine — there is no glycine equivalent elsewhere.

Same for every dose

Drug Formulation	Semaglutide/B-12/Glycine 3mg/0.5mg/5mg per mL inj
Allow Substitution	Yes — select Allow Substitution
Unit	ml
Reason for Compounding	N/V mitigation & flexibility
Pharmacy Instructions	Bill to KORB Health Group and ship to the patient. Custom Rx for N/V mitigation, dosing flexibility, and added B-12 and glycine.

0.3 mg — 4-week

Name	PREMIER – Glycine Semaglutide 0.3 mg – 4-Week Supply
Quantity	1
Refill	0
Days Supply	28
Patient Instructions	GLYCINE – INJECT 0.3 MG SUBCUTANEOUSLY ONCE WEEKLY AS DIRECTED FOR 4 WEEKS. Discard 28 days after first use.

0.3 mg — 8-week

Name	PREMIER – Glycine Semaglutide 0.3 mg – 8-Week Supply
Quantity	2
Refill	0
Days Supply	56
Patient Instructions	Maintenance – GLYCINE – INJECT 0.3 MG SUBCUTANEOUSLY ONCE WEEKLY AS DIRECTED FOR 8 WEEKS. Discard 28 days after first use.

0.6 mg — 4-week

Name	PREMIER – Glycine Semaglutide 0.6 mg – 4-Week Supply
Quantity	1
Refill	0
Days Supply	28
Patient Instructions	GLYCINE – INJECT 0.6 MG SUBCUTANEOUSLY ONCE WEEKLY AS DIRECTED FOR 4 WEEKS. Discard 28 days after first use.

0.6 mg — 8-week

Name	PREMIER – Glycine Semaglutide 0.6 mg – 8-Week Supply
Quantity	2
Refill	0
Days Supply	56
Patient Instructions	Maintenance – GLYCINE – INJECT 0.6 MG SUBCUTANEOUSLY ONCE WEEKLY AS DIRECTED FOR 8 WEEKS. Discard 28 days after first use.

1.2 mg — 4-week

Name	PREMIER – Glycine Semaglutide 1.2 mg – 4-Week Supply
Quantity	2
Refill	0
Days Supply	28
Patient Instructions	GLYCINE – INJECT 1.2 MG SUBCUTANEOUSLY ONCE WEEKLY AS DIRECTED FOR 4 WEEKS. Discard 28 days after first use.

1.2 mg — 8-week

Name	PREMIER – Glycine Semaglutide 1.2 mg – 8-Week Supply
Quantity	4
Refill	0
Days Supply	56
Patient Instructions	Maintenance – GLYCINE – INJECT 1.2 MG SUBCUTANEOUSLY ONCE WEEKLY AS DIRECTED FOR 8 WEEKS. Discard 28 days after first use.

1.8 mg — 4-week

Name	PREMIER – Glycine Semaglutide 1.8 mg – 4-Week Supply
Quantity	3
Refill	0
Days Supply	28
Patient Instructions	GLYCINE – INJECT 1.8 MG SUBCUTANEOUSLY ONCE WEEKLY AS DIRECTED FOR 4 WEEKS. Discard 28 days after first use.

1.8 mg — 8-week

Name	PREMIER – Glycine Semaglutide 1.8 mg – 8-Week Supply
Quantity	5
Refill	0
Days Supply	56
Patient Instructions	Maintenance – GLYCINE – INJECT 1.8 MG SUBCUTANEOUSLY ONCE WEEKLY AS DIRECTED FOR 8 WEEKS. Discard 28 days after first use.

2.7 mg — 4-week

Name	PREMIER – Glycine Semaglutide 2.7 mg – 4-Week Supply
Quantity	4
Refill	0
Days Supply	28
Patient Instructions	GLYCINE – INJECT 2.7 MG SUBCUTANEOUSLY ONCE WEEKLY AS DIRECTED FOR 4 WEEKS. Discard 28 days after first use.

2.7 mg — 8-week

Name	PREMIER – Glycine Semaglutide 2.7 mg – 8-Week Supply
Quantity	8
Refill	0
Days Supply	56
Patient Instructions	Maintenance – GLYCINE – INJECT 2.7 MG SUBCUTANEOUSLY ONCE WEEKLY AS DIRECTED FOR 8 WEEKS. Discard 28 days after first use.

Pricing and charge codes

Premier — Semaglutide / B-12 / Glycine — 4-week

Program	Price	Charge code
4-week — Full price — website rate	\$269	FITSema001
4-week — Discounted rate	\$199	Operations will provide the charge code at scheduling
4-week — All others	Varies	Operations will provide the charge code at scheduling

KORB Get Fit Now Semaglutide Standard

One rate across all three categories. No per-partner variation.

Partner-funded, partially covered, and other arrangements. Highly variable and changes often. Operations confirms the figure and the code at scheduling. Do not quote a price from this tool.

Premier — Semaglutide / B-12 / Glycine — 8-week

Program	Price	Charge code
All 8-week semaglutide, every patient, every pharmacy	\$349	FITSemaMNT

No discounted rate on 8-week. Corporate, friends and family, and military patients all pay \$349 for 8-week. The \$199 rate is 4-week only.

Includes: Telehealth visit, medication, shipping and supplies. Never quote pricing to a patient without confirming with Operations.

Clinical reference — Semaglutide — GLP-1 receptor agonist

Indications

- Chronic weight management and long-term maintenance of weight reduction in adults with obesity, or overweight with at least one weight-related comorbid condition
- Improvement of glycemic control in adults with type 2 diabetes mellitus
- Reduction of cardiovascular risk in adults with type 2 diabetes and established cardiovascular disease
- Reduction of kidney disease progression and cardiovascular death in adults with type 2 diabetes and chronic kidney disease
- Treatment of noncirrhotic MASH with moderate-to-advanced liver fibrosis (F2-F3) in adults

What KORB treats

KORB treats weight loss only. Every other indication above stays with the patient's PCP or specialist - KORB does not manage type 2 diabetes, cardiovascular risk, chronic kidney disease or MASH, and does not order or monitor labs for them. A patient carrying one of those diagnoses is acceptable on the weight-loss program provided that care continues elsewhere.

Identifying the appropriate candidate

Age. Adults 18 years and older. KORB does not treat patients under 18.

BMI

- BMI greater than 25 is overweight
- BMI greater than 30 is obese

Diabetes Note. KORB treats weight loss. Patients who are diabetic or pre-diabetic are acceptable. The patient must continue diabetes management with their PCP — KORB does not manage diabetes and does not order or monitor labs for it.

Definition

Long-acting glucagon-like peptide-1 (GLP-1) receptor agonist. A 31-amino-acid analogue with roughly 94% homology to human GLP-1, acylated with a C18 fatty diacid that binds albumin and extends the half-life to about one week, allowing once-weekly dosing. FDA-approved as Ozempic and Rybelsus for type 2 diabetes and as Wegovy for chronic weight management.

Mechanism

- Agonises the GLP-1 receptor.
- Glucose-dependent insulin secretion from pancreatic beta cells.
- Suppression of inappropriate glucagon release.

- Slowed gastric emptying, which prolongs satiety.
- Direct action on hypothalamic appetite centres, principally the arcuate nucleus, reducing hunger and energy intake.
- Insulin release is glucose-dependent, so hypoglycaemia risk is low as monotherapy.

Clinical evidence

- Strong randomised evidence — for the BRANDED product.
- STEP 1: mean weight loss near 15% of body weight at 68 weeks, against roughly 2.4% on placebo.
- SELECT: significant reduction in major adverse cardiovascular events in adults with overweight or obesity and established cardiovascular disease, without diabetes.
- Weight regain is common after discontinuation. Raise this at the start rather than letting it become a later surprise.

Absolute contraindications

- Personal or family history of medullary thyroid carcinoma (MTC)
- Multiple endocrine neoplasia syndrome type 2 (MEN2)
- Known hypersensitivity to semaglutide or any component of the formulation
- Pregnancy, breastfeeding, or planning pregnancy

Cautions

- History of pancreatitis — evaluate carefully before initiating
- Gastroparesis or other significant gastrointestinal motility disorder
- Active gallbladder disease or prior gallbladder-related surgical complications
- Severe renal impairment (eGFR below 30 mL/min/1.73 m²)
- Diabetic retinopathy — rapid glycaemic improvement can transiently worsen it
- History of suicidal ideation or active depression — no causal link has been established in regulatory review, but monitor and ask

Medication interactions

- Delayed gastric emptying can alter absorption of concomitant oral medication. Use caution with narrow-therapeutic-index drugs such as levothyroxine and warfarin.
- Insulin and sulfonylureas: additive hypoglycaemia risk. Dose reduction of the concomitant agent is often required. Coordinate with the prescribing PCP.
- Alcohol may increase hypoglycaemia risk and worsen gastrointestinal side effects.
- ANAESTHESIA AND PROCEDURES: delayed gastric emptying raises aspiration risk under sedation. HOLD for 7 days (one week) before an elective procedure requiring sedation, or for whatever longer period the anaesthesia team or the surgeon's office requires - their instruction takes precedence over this one. Tell the patient to disclose GLP-1 use to any surgeon, proceduralist or anaesthetist.
- Other GLP-1 receptor agonists: do not combine. Concomitant use is not recommended.

Side effects

COMMON

- Pain, redness or swelling at the injection site
- Nausea and vomiting
- Diarrhea
- Stomach pain
- Constipation
- Low appetite
- Headaches
- Dizziness

LESS COMMON

- Pancreatitis
- Gastroparesis
- Bowel obstruction
- Gallstone attacks

Monitoring

- Weight and BMI at each visit
- Ask about blood pressure — monitored by the PCP, not measured by KORB
- Ask about recent HbA1c or glucose if diabetic — ordered by the PCP, not by KORB
- Gastrointestinal tolerance, especially through dose escalation
- Abdominal pain that is severe or radiates to the back — assess for pancreatitis
- Right-upper-quadrant pain — assess for gallbladder disease
- Mood and mental health
- Lean mass preservation — protein intake and resistance training
- Injection site and adherence

Patient counseling

Semaglutide works on the same pathway as a hormone your body already makes. It slows how quickly your stomach empties and reduces appetite signalling, so you feel full sooner and stay full longer. Nausea is the most common side effect and is usually worst in the first days after a dose increase. This is a compounded preparation rather than the brand-name product. Weight tends to come back if the medication stops, so we should talk about this as a long-term plan rather than a short course. Tell any surgeon or anaesthetist that you are taking this.

Escalation and discontinuation

HARD STOP

- Suspected pancreatitis — severe persistent abdominal pain, often radiating to the back, with or without vomiting. Stop the medication and evaluate.

- Hypersensitivity or anaphylaxis-type reaction.
- Pregnancy identified or confirmed.
- Medullary thyroid carcinoma or MEN2 identified at any point.
- Persistent severe vomiting with dehydration or acute kidney injury.

FLAG AND REASSESS

- Gastrointestinal side effects that do not settle within two weeks of a dose increase — hold at the current dose or step back rather than pushing forward.
- Right-upper-quadrant pain, or gallstone symptoms.
- Rapid or excessive weight loss, or loss of lean mass.
- New or worsening depression, or any mention of suicidal thoughts.
- Worsening retinopathy in a diabetic patient during rapid glycaemic improvement.
- Plateau with no further response at the maximum tolerated dose — reassess the plan rather than continuing indefinitely.
- Patient scheduled for surgery or a procedure requiring sedation — coordinate holding.

Chart attestation

Patient counseled on semaglutide as a GLP-1 receptor agonist for chronic weight management, including that a compounded preparation is being dispensed rather than the FDA-approved branded product. Mechanism, expected time course, gastrointestinal side effects during titration, pancreatitis and gallbladder warning signs, the need to disclose GLP-1 use before any procedure requiring sedation, likelihood of weight regain on discontinuation, and the monitoring and follow-up plan were reviewed. Patient verbalized understanding and consented to proceed.

ICD-10

Note. DOCUMENTATION ONLY — NOT BILLING. Select the code matching the documented presentation.

PRIMARY

- E66.9 — Obesity, unspecified
- E66.01 — Morbid (severe) obesity due to excess calories
- E66.3 — Overweight

SECONDARY

- Z68.— — Body mass index (add the matching BMI code)
- Z71.3 — Dietary counseling and surveillance
- Z72.3 — Lack of physical exercise

Clinically reviewed

Reviewed and signed off by Donald Stevenson, PA-C (Director of Clinical Operations and Lead Provider) on 2026-09-06, against korb-glp1-data.js v2.13.

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Questions and escalation

- Pharmacy or shipping — Operations
- Charge codes and billing — Operations
- Clinical protocol — Clinical Director
- Corrections to this document — Clinical Operations

Do not improvise

- Do not substitute an unavailable pharmacy
- Do not alter quantity, refill or days supply
- Do not paraphrase patient instructions
- Do not quote pricing without Operations

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